# Pembrolizumab 200 mg · every 3 weeks · lung cancer

**Lung cancer information** This patient suite covers the two lung indications described here. The shared clinical protocol also lists other cancers, which need their own patient information.

**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Pembrolizumab 200 mg every 3 weeks

**Active medicines:** pembrolizumab

**Emetogenic risk:** Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

### Additional phase-specific support

- No routine antiemetic prophylaxis or routine infusion premedication is usually required. Use an individual/local symptom-relief prescription if needed.
- Carry the immunotherapy alert card and report possible immune symptoms promptly during and after treatment.
- Blood count, renal/liver profile and glucose before each cycle; thyroid every 3–6 weeks, cortisol if clinically indicated.
- Effective contraception during treatment and at least 4 months after the last dose.
- If treatment is paused because of an immune side effect, your team checks recovery and any steroid or hormone plan before deciding whether it can restart. Do not change steroids or restart treatment yourself.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00455 v16 · pNCCP00455 pp5–7 · NCCP00455 v16 supportive care

Sources checked 2026-10-29 · SACTCheck v0.84.26 · Regimen-level drafts; prescriber and oncology-pharmacy verification required